

Menopause: Guidance for practice

Top Ten Tips

- 1** Everyone should be able to access advice on how they can optimise their menopause transition and the years beyond. There should be an individualised approach, with particular reference to lifestyle advice, diet modification as well as discussion of the role of HRT.
- 2** In those aged 45 years and over, the diagnosis of perimenopause or menopause should be considered based on symptoms alone, without confirmatory blood tests.
- 3** HRT should be offered as first line treatment for symptoms related to the menopause, including vasomotor symptoms and low mood/anxiety, after discussing the short-term and longer-term benefits and risks.
- 4** The decision whether to take HRT, dose and duration of its use should be made on an individualised basis after discussing the benefits and risks with each patient. This should be considered in the context of the overall benefits obtained from using HRT including symptom control and improving quality of life, as well as considering the bone and cardiovascular benefits associated with HRT use. No arbitrary limits set on age or duration of HRT use.
- 5** Estrogen alone HRT is associated with little or no change in the risk of breast cancer, while combined HRT can be associated with a small increase in the risk of breast cancer. However, this risk is low in both medical and statistical terms and should be taken in the context of the overall benefits obtained from using HRT. These include improved quality of life, as well as cardiovascular and bone protective effects.
- 6** Transdermal rather than oral HRT should be considered in those who are at increased risk of VTE, including those with increased BMI.
- 7** Vaginal estrogen should be offered to women with genitourinary symptoms (including those on systemic HRT) and treatment can be continued for as long as needed to relieve symptoms.
- 8** If HRT with adequate estrogen intake has not been effective, testosterone supplementation can be considered in menopausal women with low sexual desire.
- 9** Women with Premature Ovarian Insufficiency (POI) and early menopause should be advised to continue HRT until at least the age of natural menopause.
- 10** Referral to (or seeking advice from) a specialist menopause service should be considered if:
 - treatments do not improve menopausal symptoms
 - there are ongoing troublesome side effects with treatment
 - there are contraindications to HRT
 - there is uncertainty about the most suitable treatment options for a woman's menopausal symptoms.

REVIEWED: FEBRUARY 2026

This BMS Tool for Clinicians has been developed by the medical advisory council of the British Menopause Society. It will be updated when guidance changes and/or new data becomes available.

British Menopause Society

Educates, informs and guides healthcare professionals, working in both primary and secondary care, on menopause and all aspects of post reproductive health.

www.thebms.org.uk

Copyright © 2026 British Menopause Society. All rights reserved.

Permission granted to reproduce for personal and educational use only. Commercial copying and distribution is prohibited.

Women's
Health
Concern

British
Menopause
Society